SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

# Dacarbazine — metastatic melanoma

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Advanced melanoma · NCCP 00464 · version 3 · 21-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
To control metastatic melanoma with chemotherapy, with benefit, blood counts and tolerability reviewed before each treatment.

**Schedule**
Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

**Dacarbazine**
CYTOTOXIC CHEMOTHERAPY

- ☐ Nausea and vomiting
- ☐ Low white cells, platelets or red cells
- ☐ Tiredness and appetite loss
- ☐ Infusion-site injury if medicine leaks
- ☐ Rare severe liver injury or allergic reaction

Discuss the exact diagnosis and treatment intent, expected benefit and uncertainty, alternatives, regimen-specific risks, tests, fertility, practical burdens and your preferences. This guide supports the consent conversation and does not replace it. This is single-agent dacarbazine at the NCCP 850 mg/m² dose. It is not an immune-checkpoint or BRAF-targeted regimen. Historical studies often used 1000 mg/m² and different treatment settings; their response or survival figures are not a precise forecast for this 850 mg/m² course.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00464 v3.

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